

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Lazarus E. Brandvold DOB: 1959-11-15 (age 66) Member ID: MBR-38176

Sex: M

Requesting Provider

Practice: Juniper Ridge Electrophysiology

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-10-27

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

Hospital discharge summary

ST. COLUMBA HOSPITAL - DISCHARGE SUMMARY

Admitted 09/18/2026 Discharged 09/25/2026 Attending: hospitalist service

Patient: Brandvold, Lazarus E. 66M

Principal diagnosis: septic shock due to acute cholangitis (choledocholithiasis).

Secondary diagnoses: Type 2 NSTEMI (demand ischemia) 09/20/2026; acute kidney injury, resolved; known non-ischemic cardiomyopathy.

Course: norepinephrine x 48 h, ERCP with stone extraction 09/19. hs-troponin T rose to 412 ng/L on 09/20 during hypotension and fell over 3 days. No chest pain. Cardiology consulted.

Discharged home off pressors, eating, walking the halls.

Hospitalist: Dr. Grete Oyelowo-Sand

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Inpatient cardiology consultation

ST. COLUMBA HOSPITAL - CARDIOLOGY CONSULT

09/21/2026 Brandvold, Lazarus

Asked to see for troponin elevation.

No ischemic symptoms. ECG: sinus tachycardia, no new ST changes or Q waves compared with 2025. Bedside echo: global hypokinesis unchanged from baseline, no new regional wall motion abnormality.

Assessment: acute myocardial injury due to septic shock. Does NOT meet the universal definition for myocardial infarction (no symptoms, ECG or imaging evidence of ischemia). No ischemic work-up needed. Coronaries normal on 2024 cath.

Dr. Mateo Achterberg-Lindo, Cardiology

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EP clinic note

JUNIPER RIDGE ELECTROPHYSIOLOGY

Clinic visit: 10/01/2026

Mr. Brandvold, 66

Known NICM since 2024 (normal coronaries on cath 2024). GDMT since 02/10/2025.

Recovered fully from the September sepsis admission. Back to walking his dog; short of breath on stairs and long walks. NYHA II. No syncope.

TTE 10/01/2026: LVEF 26%.

Plan: primary prevention ICD as planned before his illness. SDM today with our NP. Admit 10/27.

Dr. Solveig Mankiller-Reyes

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Echocardiogram report

JUNIPER RIDGE - TTE

10/01/2026 BRANDVOLD, LAZARUS E

LVEF 26% (biplane). LV dilated, global hypokinesis. Mild MR.

Compared with 03/2026 (LVEF 27%): unchanged.

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Shared decision-making note

JUNIPER RIDGE EP - SDM ENCOUNTER

10/01/2026 Lazarus Brandvold (wife present)

Clinician: Folake Henderson-Ruiz, NP

Evidence-based ICD decision aid (video and handout) reviewed together.

Benefit, risks, shocks, infection, deactivation and alternatives discussed. He chose the ICD.

F. Henderson-Ruiz NP

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Medication list

Brandvold, Lazarus - medicines 10/01/2026
sacubitril/valsartan 49/51 BID from 02/10/2025
metoprolol succinate 100 mg from 02/10/2025
spironolactone 25 mg from 02/10/2025
empagliflozin 10 mg from 04/2025
ursodiol (post-ERCP) from 09/2026

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Admission plan

JUNIPER RIDGE EP - ADMISSION PLAN 10/02/2026

Brandvold, Lazarus E. MBR-38176 CPT 33249 Admit 10/27/2026 inpatient

Expected LOS: 2 midnights. Reasons: recent septic shock with AKI (renal check after contrast); type 2 diabetes on insulin; obesity BMI 37.

No PCI or CABG ever.

Limiting conditions: none. Clinically stable, cognition intact, no terminal illness, no AF.

Dr. Solveig Mankiller-Reyes

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